



Treating Bacterial Mastitis

Bacterial mastitis represents a progression from ductal narrowing and inflammatory mastitis to an entity necessitating antibiotics or probiotics to resolve. Common organisms in lactational mastitis include *Staphylococcus* (e.g., *S. aureus*, *S. epidermidis*, *S. lugdunensis*, and *S. hominis*) and *Streptococcus* (e.g., *S. mitis*, *S. salivarius*, *S. pyogenes*, and *S. agalactiae*). Despite the common perception that yeasts cause “candida mastitis,” no scientific evidence exists to support this diagnosis, and the sterilization of pump parts or infant toys is not recommended to “eradicate” yeast.

Bacterial mastitis is not a contagious entity. It neither poses a risk to the infant, nor requires an interruption in breastfeeding. There is no evidence to support poor hygiene as a cause of bacterial mastitis or the need for routine sterilization of pumps. Handwashing before milk expression and basic pump cleaning practices should be followed.

Empiric Antibiotic Management

First Line

- ◆ Dicloxacillin or flucloxacillin 500 mg QID for 10 – 14 days
- ◆ Where dicloxacillin and flucloxacillin are unavailable, cloxacillin can be used; however, oral bioavailability is more variable with cloxacillin. All drugs have low Relative Infant Dose of the drug.
- ◆ Cephalexin 500 mg QID for 10 – 14 days
 - ◆ Broader coverage including gram negative rods; does not need to be taken separately from meals

Second Line

- ◆ Clindamycin 300 mg four times daily for 10 – 14 days
- ◆ Trimethoprim-sulfamethoxazole DS BID for 10 – 14 days
 - ◆ This isn't recommended for mothers of children with G6PD deficiency. Use with caution in mothers with premature infants or infants with hyperbilirubinemia, especially under 30 days old.

References

Mitchell, K. B., Johnson, H. M., Rodriguez, J. M., Eglash, A., Scherzinger, C., Widmer, K., Berens, P., Miller, B., & the Academy of Breastfeeding Medicine. (2022). Academy of Breastfeeding Medicine clinical protocol #36: The mastitis spectrum, revised 2022. *Breastfeeding Medicine*, 17, 360-376. <https://www.liebertpub.com/doi/full/10.1089/bfm.2022.29207.kbm>